

Heritage Valley Beaver

1000 Dutch Ridge Rd, Beaver, PA 15009

DISCHARGE SUMMARY Document #: DIS-010

Patient & Admission

Name	Evan Ashley
Date of Birth	1975-07-14
Phone	878-555-7527
Address	148 Eric Track Beaver, PA, 15009
MRN	MRN-543039
Admit Date	2026-04-19
Discharge Date	2026-04-22
Attending	Nicholas Davis, MD
Attending NPI	2624731784

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	HSB784 801845

Primary Discharge Diagnosis

Acute on chronic systolic heart failure

ICD-10: I50.21, I25.10

Hospital Course

The patient presented to the emergency department with acute dyspnea and orthopnea, consistent with acute decompensated heart failure. She reported a three-day history of progressive shortness of breath, lower extremity edema, and weight gain of approximately eight pounds over one week. Physical examination revealed bibasilar crackles, elevated jugular venous pressure, and 2+ peripheral edema. Initial BNP was markedly elevated at 1,240 pg/mL. Chest radiography demonstrated pulmonary edema with bilateral infiltrates. Transthoracic echocardiography confirmed reduced ejection fraction of 28%, consistent with her known ischemic cardiomyopathy. Troponin and serial electrocardiograms were reassuring, with no evidence of acute coronary syndrome. The patient was admitted to the telemetry unit for diuresis and optimization of her heart failure regimen.

During her three-day hospitalization, she received aggressive intravenous diuretic therapy with furosemide, achieving net negative fluid balance of 4.2 liters. Her respiratory status improved significantly, with resolution of orthopnea and dyspnea on exertion. Renal function remained stable throughout admission with creatinine ranging from 1.1 to 1.3 mg/dL. Electrolytes were monitored closely and repleted as needed. Her home medications were reviewed and optimized; her lisinopril was uptitrated to 20 mg daily and carvedilol was increased to 25 mg twice daily. She was transitioned to oral diuretics prior to discharge with a target daily

furosemide dose of 40 mg.

The patient demonstrated excellent clinical response to therapy with resolution of pulmonary edema, normalization of vital signs, and return to baseline functional status. She was discharged home in stable condition with close outpatient cardiology follow-up scheduled within one week. Discharge instructions emphasized strict sodium and fluid restriction, daily weights with parameters for contacting her cardiologist, and medication adherence. She was counseled on recognizing early signs of decompensation and instructed to seek immediate care if dyspnea, orthopnea, or significant weight gain recurred.

Procedures Performed

IV diuresis with furosemide	—
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Medication Changes

Spironolactone 25 mg daily	STARTED
Sacubitril/valsartan 49/51 mg BID	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	11 days
Urgency Tier	CRITICAL
Clinical Flags	BNP elevated at discharge

Dictated and electronically signed by:
Nicholas Davis, MD
NPI: 2624731784
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